

Adult Cardiac Arrest – Initial Care

EMT/AEMT

Patient Unresponsive, Pulseless and Apneic

- CPR – proper depth, with full recoil, at rate 100-120 minute.
- Ensure high quality CPR and rotate compressor every 2 minutes or 5 cycles. No Pauses.
- Ventilate 10 times min. (every 6 seconds) with 400 ml tidal volume (two finger BVM squeeze technique)
- Stop CPR only to analyze rhythm (if the AED cannot analyze through compressions) and deliver shock. Re-start compressions immediately after shock delivered
- If unable to ventilate effectively use advanced airway device / SGA

Shock Advised – inform Med Leader patient is in shockable rhythm, if EMS is not on scene

- CPR while Charging
- Shock (AED)
- Continue CPR,
- Ventilations with BVM
- Place supraglottic airway (SGA) immediately

No Shock Advised

- Continue CPR with ResQ CPR system
- Ventilations with BVM
- Consider Advanced Airway / SGA placement

Paramedic

- Leave SGA in place and appropriate airway/oxygenation is maintained can leave in place
- Intubation if supraglottic airway (SGA) insufficient or definitive airway preferred then (2 initial attempts) – without CPR interruption
 - Capnography confirmation

Lightning strike and drowning patients

Airway/ventilations are of equal importance to chest compressions.

Provide adequate ventilation at 30:2 rate until intubated and then 100 compressions/min with ventilation every 6-8 seconds.

Continue high quality CPR and ventilation until:

- Return of spontaneous circulation (ROSC) is achieved
- Resources are exhausted
- The scene becomes unsafe for medical and rescue personnel
- Advanced care and transport unit arrives to assist